

Costa Rican Healthcare Sector Debt Reform

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Introduction

The healthcare system of Costa Rica is composed of a multitude of agencies within the Costa Rican State. Some of these institutions are autonomous, and bureaucratically independent from the Costa Rican President or Congress, while others are directly controlled by the Executive Power. Throughout this paper, we will first discuss the inception of the Costa Rican Social Security Fund—the most important institution that this sector has—the debates and reforms which impacted it, and its interaction with other parts of the Health sector such as the Ministry of Health, the Ministry of Labor and Social Security, the Medics Union, FODESAF, etc. This history will also include analysis offered by reformers of the time in addition to my own, the analysis of Panamá's own CSS and Ministry of Health, and how failed reforms failed reforms can warn about future dangers to Costa Rica. Finally, a discussion of possible reforms to these issues, that fit the status quo of the system, will be presented with the goal of strengthening the institution that has cared for millions of Costa Ricans since its birth more than 84 years ago.

Past (1940-1986)

The Election of President Calderon Guardia & The Birth of the CCSS (1940-1948)

The 1940 election results saw Dr. Calderon Guardia win the presidency. During the campaign, Dr. Guillermo Padilla Castro, and President Calderon Guardia, his close friend, secretly discussed the need to create a Social Security Institution in the country. After the election, President Calderon Guardia sent Dr. Padilla Castro to Chile in 1941 to attend the Social Security Conference held in Santiago, with the goal of drafting a bill to create what we know as the Costa Rican Social Security Fund (CCSS) (de la Cruz 1995a) & (Diario de Costa Rica 1940).

Thanks to the cooperation of the Chilean, and Peruvian governments, Dr. Padilla Castro was able to finish his draft of the bill in just six months, and presented it to President Calderon Guardia in mid-1941. In particular, Dr. Padilla Castro learned about the successes and failures of Chile's social security institution since its creation in 1925. He understood the necessity for the prospective institution to be universal—in order to avoid it becoming antagonistic to the population—and the need to give the institution a robust economic base. The International Labor Organization's (ILO) office in Chile served an important role by giving feedback to Dr. Padilla Castro during his time in Chile. Throughout this process, President Calderon Guardia also sought advice from Tomás Soley Güell¹, Horacio Castro², and Raúl Gurdíán³. Of the three men only Soley Güell did not support the bill, arguing that the institution “would be in grave danger of hitting the wall of economic impossibilities” (de la Cruz 1995a) & (de la Cruz 1996).

Dr. Padilla Castro's original draft was edited after pressure from Congressman Ernesto Martén⁴. Articles 9 and 15 of Law N°17 contained these changes: article 9 stated that the Executive Power would hold control of the CCSS's budget, while article 15 dictated that the CCSS would start its duties immediately with the implementation of the medical services for Maternity and Sickness (SEM) and the insurance against old age. These reforms put great financial pressure on the first years of the CCSS, and made the institution bureaucratically dependent on the Executive (de la Cruz 1996). The only area covered during the first year of operation was San José, with only workers that paid their contributions to the Fund receiving medical services from the institution (CCSS 1961).

According to Dr. Padilla Castro, some of the financial woes of the institution were evident during 1942, the first full year of service. First, the institution was forced to begin operations without having first established its hierarchical structure, so the tracking of

debts—owed to the San Juan de Dios Hospital that gave care to those under the coverage of the institution—was poor. Second, beneficiaries were also confused by the difference in service given to them through the institution’s coverage, and the care given to poor patients who received ‘charitable’ care at the San Juan de Dios Hospital. These issues made it clear to Dr. Padilla Castro that the institution needed its own hospital, for which President Calderon Guardia provided an unfinished hospital building, which Dr. Padilla Castro and other reformers were tasked to finish, and it opened its doors in October 1943⁵(de la Cruz 1996). On October 22nd of that same year, the law that created the CCSS was reformed, and the institution was finally granted bureaucratic autonomy from the Executive power. The CCSS mandate to protect workers from the risks of disability, old age, and death (IVM), was empowered with the creation of the SEM/IVM regime, but it did not start its coverage until 1947. Under the regime implemented in 1947, workers, their employers, and the government contributed 2.5% of the total value of the workers wages for a month (up to a maximum of ₡400 CRC), for a total of 7.5% per month in “Social Charges” for the SEM/IVM (Arias et al. 2016) & (Cespedes & Paniagua 2025).

Another financial issue experienced by the CCSS in its first few years was the provision of pharmaceutical services. The CCSS allowed doctors to order medicine for their patients at private pharmacies, a cost that if left uncontrolled could threaten the institution, so a decision was made to pursue the creation of the CCSS’s own pharmacies. In these early years, many of the critics of the institution cited its lack of financial balance as their reason for disapproving its mission. Multiple attempts were made to bring an experienced actuary to help the institution organize its finances, with four of them working for the CCSS within a two year period, but experiencing difficulties, due to either inexperience in the Social Security system, or ideological differences with the heads of the institution (de la Cruz 1996).

On August 8th 1945, the CCSS and the National Board of Housing were merged together. The bill was written by then-Assistant Manager Padilla Castro, who argued that the CCSS was the only government institution that had the necessary funds to fulfill the housing needs of the country; and allowing the marginalized sectors of society to live in affordable and sanitary housing would make the population healthier, and the CCSS would be fulfilling its duty to protect the health of citizens. He would eventually retract this position in July 1948, when he advocated publicly for the Board of Housing to be remade as a separate autonomous institution (de la Cruz 1996) (Mujer y Hogar 1945), (La Tribuna 1945) & (Ley N° 148 de 1945).

In 1946, as the Social Security Fund’s finances kept weakening, Dr. Aladar Metali, a high ranking ILO official, arrived in Costa Rica to assist the CCSS leadership in solving this issue. The conclusion to which Dr. Metali arrived, was that the CCSS was only taking on “negative” insurance risk, since being affiliated to the CCSS was only mandatory for workers with salaries under ₡400 CRC (~\$71 USD) (Vindas-Chaves 2021). His recommendations were to make it mandatory for all workers to be affiliated to the CCSS, that way the CCSS would be able to balance the negative risk of the lowest earners, with those of the highest earners. The Doctors Association of Costa Rica, however, heavily opposed this, and formed the Medics Union to oppose this reform. The union feared that such a reform would put private medicine out of business, and threatened to strike⁸, if this agreement was not nullified. After the agreement was not abandoned, the strike took place, and after only three doctors were left to take care of more than 100 patients, the CCSS decided to delay this reform, putting more strain on the institution’s finances. Shortly after this strike, Dr. Padilla Castro decided to leave the institution, citing the need for the State to remove politics from the internal workings of the Fund and ensure the

institution's constitutional autonomy as one of his reasons (de la Cruz 1996). In 1947, the IVM pension system expanded coverage to the public sector's workers, and shortly after included administrative workers of the private sector as well. (Arias et al. 2016).

The Civil War, the National Liberation Party (PLN), and the Second Republic (1948-1956)

In 1948, the decision of the Electoral Board to certify the victory of Otilio Ulate Blanco⁹ over former President Calderon Guardia, the Victory Bloc's¹⁰ candidate, who accused Ulate Blanco of fraud, so the election was nullified by Congress on March 1st. That same day, the government had the house of Dr. Carlos Luis Valverde Vega under police surveillance, since it was the headquarters of Ulate Blanco's party. Dr. Valverde Vega, and two police officers died in the shooting of that day¹¹. 11 days later Jose Maria Figueres Ferrer organized the National Liberation Army, engaging the Costa Rican military forces in combat until April 19th, when the government surrendered (Espiritu del 48 n.d). For the communist forces to surrender their weapons, Secretary General Manuel Mora Valverde wanted the insurgent forces to assure them that the CCSS—and the other institutions that formed the Social Warranties¹²—would not be dismantled, to which Figueres Ferrer agreed. All of the reforms that took place under the Calderon Guardia and Picado Michalski administrations were thus preserved. Figueres Ferrer led the Founding Board of the Second Republic, the group of elite insurgents that ruled by decree and without a Congress for 18 months, and organized elections for the new Constituent Assembly. (de la Cruz 1995a) & (Espiritu del 48 n.d).

In 1956, under President Figueres Ferrer, a new reform expanded medical services to some family members of insured workers: his wife or partner, children under the age of 12, and parents dependent on this worker. The next year, issues within the management structure of the CCSS led to Dr. Fernando Trejos Escalante, the Manager of Medical Services of the CCSS, leaving his position, citing a lack of response—from the Fund's Board of Directors—regarding his request for the creation of the Assistant Manager of Medical Services position. Upon hearing of this, the Medics Union agreed that none of its members would replace Dr. Trejos Escalante unless an Assistant Manager was hired, so the Fund eventually did (de la Cruz 1995a).

The Universal Healthcare System and the Crisis (1958-1986)

In 1958, the financial situation of the Fund had gotten much weaker, so incoming President Mario José Echandi Jiménez and his cabinet had to react. At this point in time, coverage had yet to be expanded to additional citizens that could contribute financially, and the Fund was funding a much wider range of medical services as a consequence of the service expansion of 1956. The recommendations that Dr. Metali of the ILO made back in 1946 became impossible to delay any longer. Since the Medics Union blocked the coverage increase—that would have included “positive” risk, associated with higher wage-earners contributing more funds—back then, the CCSS had only been increasing expenditures and not revenue—obligatory contributions were still kept at the ₡400 CRC monthly ceiling, which most likely meant that revenue shrank when accounting for inflation. Additionally, the Costa Rican government had not been paying in full its SEM/IVM contributions to the CCSS, so the government had started accumulating debts with the institution at this point in time (de la Cruz 1995a).

To solve this problem, Minister of Labor Franklin Solorzano presented to congress two major pieces of legislation to solve the financial woes of the institution. The first of them increased the contribution ceiling to include monthly salaries of ₡1000 CRC, while the other intended to make other autonomous institutions pay for a part of the SEM/IVM funds the Central Government was supposed to pay. The first bill was approved by the legislature in October 1958, while the second one faced tough opposition from PLN congressmembers, who eventually rejected it. After the latter's failure, the President authorized the Ministers of Finance and Labor to assign the revenue from cigarette sales tax to the CCSS, which after Congressional approval, was signed into law in February 1960 (CCSS 1962) & (de la Cruz 1995a).

These reforms were still not enough to cover, or ensure the continued payment of the government's duties to the CCSS, so some officials feared that the institution would face the same issue again. To solve this, in May 1961 President Echandi Jimenez signed Laws N^o 2737—that gave full control of all Social Securities (IVM and SEM) to the CCSS, and prohibited the transferring of CCSS personnel, assets, and funds for any purpose other than those under its purpose—and N^o 2738—which assigned 10% of Costa Rica's total tax revenue to be distributed to the CCSS for each year¹³, and set a limit of ten years for the CCSS to universalize coverage for citizens (CCSS 1962) & (de la Cruz 1995a). The directives of Law 2737, however, still allowed for the CCSS to invest a part of its IVM budget—as directed in Law 2406 of 1959¹⁴—in mortgage, agricultural, and cattle-raising loans; and credit for workers and employers. The CCSS was also still allowed to loan funds to—and buy government bonds from—other government institutions such as the National Bank of Costa Rica (BNCR), the University of Costa Rica (UCR), the Bank of Costa Rica (BCR), the Anglo-Costa Rican Bank¹⁵, the Costa Rican Institute of Electricity (ICE), the Ministry of Health, among others. In 1962, 90% of the Social Security Fund's total active assets were long-term investments, according to an internal audit (CCSS 1962).

Through the end of the 1960s, three other major events seen by the institution were, the Medics Union strike of 1965, the 1966 change to allow student doctors to be taught at CCSS hospitals by their respective faculty, and in 1969, when the Mexico Hospital opened its doors. The strike of 1965 was undertaken by doctors with the goal to increase wages, have the Minister of Health be the representative of the Executive in the Board of Directors of the CCSS, and prohibit the naming of personnel to more than one concurrent administrative position (Ley N^o 3671 de 1966). The labor undertaken to plan and build a hospital was new to the CCSS, so it requested assistance from a multitude of social security agencies across the world, and in honor to the Mexican Social Security Institute's donation of hospital building blueprints, and economic resources, the hospital received its name (Hospital Mexico 2004) & (de la Cruz 1995a).

In 1970, former President Figueres Ferrer returned to the presidency after receiving 54.7% of the vote. This administration decided to take on the mission to enact the universalization of the CCSS coverage, which past administrations had not undertaken. The reforms of 1961 had set a 10 year limit, but in the two previous administrations no progress was made, and only one year was left to fulfill this responsibility. The steps taken by Jose Luis Orlich

Bormarlich, the Minister of Health, and Guido Miranda Gutiérrez, the Assistant Manager of Medicine at the CCSS, to extend coverage nationwide included the creation of a National Healthcare Plan between the Ministry and the CCSS to coordinate actions (starting in 1971); the complete removal of the SEM/IVM contribution ceiling¹⁶ (1971-1975); turning over of hospitals¹⁷ to the CCSS (1974-1977); the legal monopolization of all prevention responsibilities, not assigned to the CCSS, to the Ministry of Health (1973), etc. (Ley N° 5349 de 1973) & (de la Cruz 1995a).

The expansion of coverage proposed by the Figueres Ferrer administration included health services for all Costa Ricans, regardless of employment status, declaring healthcare a public interest protected by the State. On the expanded hospital management, the turning over of hospitals to the Fund depended on the CCSS approaching the managing institution to start the process, and assigning budgets to meet the financial needs of these hospitals. The National Healthcare Plan, made by Minister Orlich Bormarlich and Assistant Manager Miranda Gutierrez, made it the goal of the CCSS to slowly take over all curative care and individualized medicine, while the Ministry would keep its role as the overseer of the Health Code, and would directly manage all programs related to public health¹⁸. The national expansion of services was to be regionalized—the CCSS would create regional hierarchies that would oversee individual jurisdictions' problems, to ensure health responses are fast and context-adjusted. The reforms also authorized the CCSS to take part in all projects that benefit the health of the insured population, through the corresponding financial and technical assistance (Ley N° 5349 de 1973) & (de la Cruz 1995a). An audit conducted by the Ministry of Labor of Spain between May and July of 1974, said the CCSS “took steps forward that place it in a very relevant position within the current framework and projection of continental [Social] Security” (de la Cruz 1995a).

These reforms, although extremely necessary and beneficial to the overall population, kept liberalizing the ways in which the CCSS could direct its spending mostly to projects that would increase the amount of negative risk that the institution took on, while neglecting the need for stronger financial bases. The balance on which reformers found themselves at the time was limited in all directions—doctor's played an important role in delaying universalization for almost 30 years; national capitalists, and other foreign interests, kept a tight hold on progressive reforms due to class interests; and worker's unions and syndicates demanded more coverage to live less stressful lives. As a result, the tax increases that should have been paired with the service expansions never took place, making these changes entirely populist in nature, while increases that did happen, were only possible after efforts from reformers.

After the election of Daniel Oduber Quiros, the PLN candidate, as president in 1974, more expansions were on the horizon for the system. In June, a Credit Commission was created within the CCSS to evaluate future investments of IVM funds, and in December, the President signed into law the creation of the Directorate of Social Development and Family Allocations (FODESAF)¹⁹. In 1975, Jenaro Valverde Marin, the first Executive President of the CCSS²⁰, announced that on top of ₡70 million CRC of the Fund's own resources, the institution would

take a ₡165 million CRC (\$20 million USD) loan from the Inter-American Development Bank (IDB) for the purpose of building 12 clinics, 3 regional hospitals, and 3 Materno-Infant hospitals. The Fund would have 35 years to pay, of which 8 would be at 0% interest, and the CCSS would only pay 2% interest over the remaining 27 years. Another important decision of Valverde Marin was the purchase of Travenol Laboratories for \$400,000 USD for the CCSS to continue expanding its pharmaceutical manufacturing operations. This move, similar to the decrease of the SEM/IVM contributions from the State from 4.5% to 0.5%, paired with the raise for employers from 7.5% to 11.5%, was opposed by the business sector, but the legal system protected these decisions from lawsuits launched by capital owners. The State Insurance, meant for those who earn less than ₡500 CRC per month; and the Independent Worker Insurance, meant for small business owners who would contribute 4% of their earnings, were created that year as well. The goal of reformers was to insure 75% of the population by December 1976, so every time someone who was not insured sought medical services from the CCSS, a socioeconomic study was done to determine if their monthly earnings were under the ₡500 CRC cutoff for the State Insurance to classify them properly (Ley N° 5662) & (de la Cruz 1995a).

After the 1978 election of the opposition candidate Rodrigo Carazo Odio, not many more expansions of service happened because Costa Rica fell into a deep economic crisis caused by the administration's inability to combat the rising oil prices of the late 1970s, which lowered exports of coffee and other goods Costa Rica depended on. To face the lack of funding for the 1981-1982 period, the Executive Power and the CCSS agreed to raise the State's SEM contributions from 0.25% to 3.25%, while keeping the same contributions for employers and workers (La Prensa Libre 1981). In 1981, the implementation of the Mixed Care System (SAM) attempted to offload costs and reduce wait times by offering free x-rays, exams, and 3 prescriptions to insurees who paid for a consultation with a private doctor. Critics raised concerns over doctors caring for private practice patients in CCSS hospitals, since it allowed doctors to use these facilities for their private practice. By the end of the Carazo Odio administration, the CCSS had used ₡800 million CRC in IVM funds to keep SEM operational, and stopped the IVM contributions of 5% to FODESAF; 1.5% to the Popular Bank (BPDC); 0.5% to the Mixed Institute of Social Assistance (IMAS); and 1% to the National Institute of Learning (INA). In May 1982, the total owed by the CCSS to them was more than ₡500 million CRC, and, due to inflation, the CCSS's assets had been reduced by 40% in value (Revelo 2015).

After the election of 1982 handed power back to the PLN, President Luis Alberto Monge Alvarez had to end the Medics Union strike. Doctors went on strike on April 27th—11 days before he assumed power—until June 7th of 1982. The Union said that the CCSS had broken their promise to raise their salaries in mid-1981, since the collective compensation system agreed upon in 1979 was not created. The resulting Law N° 6836 of 1982, included future automatic wage increases for doctors. The new Congress formed a commission that looked at the CCSS finances. This commission delivered a majority report, and two minority reports after two years, but no action was taken. For the period of 1983-1985, the spending of the CCSS was severely restricted, affecting hiring, medication reserves, scholarships, etc. In 1982, distributions of the

SEM/IVM were 14% for employers, 1.5% for the State, and 8% for workers (Revelo 2015), (Céspedes & Paniagua 2025), (Arias et al. 2016) & (de la Cruz 1995a).

Present (1986-)

Continued accumulation of debt by the system

Since the end of President Monge Alvarez's administration, continued weakenings of the system have been institutionalized through impositions of different International Financial Institutions (IFIs) such as the IMF, IDB, World Bank (WB), etc.; pressure from business groups, or through avoidance of duties. The tax reforms enacted from impositions of the aforementioned IFIs have lowered tax rates for the wealthiest, created uneven taxation rates that unequally affect the working class, and weakened autonomous governmental institutions by forcing them to pay income taxes—inducing privatization (Ulate 2000). These changes have continued to pressure the healthcare system, and have required additional measures meant to solidify their finances. Reforms have largely refused to tax the wealthiest sectors of Costa Rican society, but have resulted in increases for the working class. The system of Social Charges has only continued to endure increased evasion of payments, from both the private sector and, most importantly, the central government. To illustrate this, total debt from independent workers and employers to the CCSS grew from ₡59.969 billion CRC in 2012, to ₡99.963 billion CRC in 2015. Most of this tax evasion stems from the 8% growth of informal employment between those years, since they account for ₡52 billion CRC of the 2015 total number, more than double of their 2012 figure (Arias et al. 2016). For the government, in 2011, it owed the CCSS ₡421.276 billion CRC, with 75% of it having no formalized payment scheme to relieve the CCSS (Badilla & Nazareth 2018).

The increase in this debt has only worsened as the mechanisms used in the past by the Executive power have remained unchecked. As discussed in the prior section, the government continues finding different ways to circumvent limits on the investment of IVM funds, an example of this kind of behavior is the use of government bonds as payment for debts with the SEM, now renamed Health Insurance (RS), system. Because the RS needs to use revenue to pay immediate needs of its insured population, the CCSS has resorted to the use of IVM reserves to buy these bonds, as a way to transfer liquidity to the RS—a practice that has worsened since the lowering of IVM reserve requirements in 1986-1990 under the presidency of Oscar Arias Sanchez. The liquidity issues are worsened by cash heavy labor payouts, such as the payment of full wages for sick days, layoff (firing, retirement, or death, in which case the family of the deceased receives the payment) payments of up to 20 consecutive years, and improper tracking of the number of hours worked by employees (Badilla & Nazareth 2018) & (Rodriguez 2009).

As of December 2024, audits conducted by the General Comptroller of the Republic (CGR) revealed the total debt of the State of Costa Rica to the CCSS had reached nearly ₡4.2 Trillion CRC (\$8.4 billion USD), approximately 8.8% of Costa Rica's total GDP for 2024. More specifically, the issue has been made worse by the lack of consistent accounting between the CCSS, the Executive Power, and FODESAF, which disagree on the amount owed by the State to the Social Security Fund. Per the CGR, the Ministry of Finance and FODESAF only recognize approximately ₡951 billion CRC of the total owed to the Fund—₡765 billion CRC from medical services corresponding to FODESAF, and ₡186 billion CRC directly owed by the Executive Power (El Observador 2025). The accounting from the Central Government raises concerns, due to its failure to prevent double counting the transactions between different institutions within its control, failing to meet the standards set by the International Public Sector Accounting Standards Board (IPSASB), of which Costa Rica is a signatory (CGR 2025).

Comparative lessons from Panamá

Although the Costa Rican case is extremely unique in the way its Social Security is set up, many similarities exist between it and other countries that face financial pressures on their own health sectors. The focus of this section is on the IVM pension system, the most critical of the CCSS's financial troubles, as its investments fund a vast network of spending across the healthcare system and the entire Costa Rican economy. Estimates from 2022 show that the IVM system would have to use its reserves in 2041 to pay beneficiaries—since its costs will exceed its income—and will lead to a complete depletion of funds in 2047 (Semanario Universidad 2024). This issue, although seemingly far, has been in constant need of intervention since the early 2000s, when different authors estimated the total depletion of reserves to happen sometime between 2022 and 2028 (Badilla & Nazareth 2018). One of the reforms enacted to solve this was the 2005 bill that set increases in contributions from employers, workers, and the State, starting in 2009. This reform gave an increased breathing room for the system, but it is still just a “band-aid” solution, since the core problem of tax evasion remains a threat for the future (Cespedes & Paniagua 2025).

Panamá's pre-2005 pension system had a very similar structure to Costa Rica's, the Social Security Fund of Panamá (CSS) had control of the country's IVM system, which was the unique public pension system with very similar goals to that of Costa Rica, also receiving contributions from employers, workers, and the State. The contributions, according to data from 1999, were as follows: for Panamá, 6.75% from workers, 2.75% from employers, and the government would contribute all revenue from the tax on alcohol; for Costa Rica, 2.5% from workers, 4.75% from employers, and 0.25% from the State. Both countries kept very similar population age distributions and percentages of coverage (Lora & Pagés 2000). 2004 data also showed that Panamá had 1.7% more of their population contributing to the IVM (Arza 2009). After the U.S invasion of Panamá in 1990, the country sought loans from IFIs to rebuild the damages caused by the invading army's occupation. The two most important were the World Bank's 1997 imposition—to slowly privatize all parts of some coverage regions of the CSS—and the IMF's 1999 demands—to create a new public-private pension system that would solve the IVM's “deficiencies” (Arias 2021). In 2005, Panamá signed into law the reform to its pension system, creating the Personal Accounts Mixed Subsystem²¹, which would limit funds available for investment by separating a part of worker's contributions to be placed in a personal account that intended to guarantee the availability of a minimum pension for beneficiaries (Lorenzo et al. 2025). After 9 years of no additional demands—and some satisfactory comments—on the pension system by the IMF, new impositions were included in a 2015 report on Panamá's pension reform of 2005. In the document, the IMF argues that Panamá's 2005 reforms did not go far enough, and that the same structural problems of 2005 remained in it. The IMF also warned that the 2005 Pension System's funds would start having to use its mandated reserves to pay retirees in 2016, until running out of funds entirely by 2027. After this, impositions were made to merge all funds from the different subsystems, and limit the types and amounts of investments that the fund can make in the future to increase revenue (Arias 2021).

In reality, the depletion happened faster than initially expected, and in 2024 the pension fund's reserves were officially emptied, resulting in an increase to 70% for employers contributions in early 2025, and the start of the merger process between the Ministry of Health and the CSS on November 26th (La Prensa 2024a), (La Prensa 2024b) & (La Prensa 2025). The changes to the system deteriorated it massively by narrowing down available reserves, keeping the Central Bank of Panamá and the Savings Fund²² with their hands tied, through the limits imposed on investments. These limitations also did not increase the returns on these investments, instead the Panamanian pension fund lost money during half of the years in the 2006-2012 period (Universidad de Panamá 2021). After the last trimester of 2012, new information systems used by the Government of Panamá distorted the fluctuations of the fund, but using data from three different sources we find that between 2019-2023, the pension fund decreased (relative to GDP) from 1.2% to 0.9%, while GDP increased from \$69.78 billion USD to \$83.32 billion USD—implying an estimated loss of \$87.4 million USD for the pension system (OECD 2015), (OECD 2025b) & (World Bank 2025).

Future

Addressing the Debt Crisis and the Deterioration of the Healthcare System

From the Panamá system, it is clear that too many regulations, on the investment of these funds, might lead to the system's demise. While the costs incurred by the Panamanian system in 2005 to create the new model did have some effect on its fate—it is more probable that the impositions made by the IMF were the cause. The limits made it so that Panamá (as of 2019) had to place more than 60% of its investment funds in savings accounts at banks, limiting their ability to diversify their investments, and increasing the State's dependence on IFI loan disbursements. Costa Rica also has an issue of diversification of investment too—but not in the same type of investment—with more than 60% used to hold debt of other public institutions of Costa Rica (Mesa-Lago 2021). The current state of the IVM system opens up the possibility for pensions to lose a massive amount of value, similar to the way in which it happened under President Carazo Odio, if any of the other autonomous institutions of the State faced liquidity issues. The diversification of investment into initiatives for more equitable development, such as affordable housing, public transit, etc., should be pursued by the institution to avoid the homogeneity in investment that currently exists. These investments could also be tied to an individual's affiliation to the institution (in terms of housing), and it would also be a possibility to make use of the socio-economic studies that the Fund undertakes when analyzing prospective enrollments. For public transit, it had already been done previously in the early 1960s—when the Fund lent to the nationalized pacific railway for purchases of new trains (CCSS 1962). This would also offer the chance to the Costa Rican government to avoid undertaking more expensive loans from IFIs, by opening the possibility to reduce the amount the State loaned from them for important development projects.

Although the diversification of the reserves is an exciting prospect, especially for a nation that needs to continue developing its infrastructure and reducing economic inequalities across the entire national territory, the Costa Rican government needs to resolve its differences in

accounting across institutions. To solve this, a joint debt-repayment roadmap between the Ministry of Finance, FODESAF, and the CCSS—with the technical assistance of the CGR—needs to be created. As pointed out before, the CGR has discovered many differences in accounting, duplicate sums of revenue, and improper valuations of assets, which would also avoid wasting more time, and resources, on duplicate audits if done jointly (CGR 2025). The increased transparency in accounting would consequently allow the CCSS to make use of legal measures, to ensure the government remains accountable to its finances. Additionally, the processing of File 24.025²³—a bill, not yet voted or debated, that aims to eliminate the position of the Executive Presidency of the CCSS—should be prioritized by Congress and the Executive Power²⁴. The elimination of this role needs to be a priority, since this role has allowed Presidents of the Republic to increase control over the autonomous institutions of the State, violating their constitutional mandate (Delfino 2025a). The additional control of the Executive has resulted in the CCSS to be the largest debt holder of the State, and even allowing the State to pay the spending (that it chooses to recognize) with bonds or other assets of value lesser than direct transactions (Badilla & Nazareth 2018). The restructuring of the Board of Directors of the CCSS, contained within File 24.025, which reforms its nine seats to be equally distributed for elected representatives of workers, employers, and the State, would democratize the decisionmaking of the institution (Delfino 2025a). In this way, welfare-reducing decisions that add unnecessary costs to the institution, such as the construction delays of the Dr. Maximiliano Peralta Jimenez Hospital in Cartago²⁵—an effort led by Marta Esquivel Rodriguez, Executive President of the CCSS—can be avoided in the future (Diario Extra 2025). Another way in which this reform helps the institution by the added representation of the labor sector, and the reduced interference of the government, is that workers now get to be directly involved in ways to better service, and would force the government to fulfill its constitutional mandate as the protector of the working class of the country. To combat tax evasion from non-government actors, the reform of evasion incentivizing laws such as those regarding banking confidentiality, should also be prioritized, expanding upon those proposed by File 24.658²⁶, which reforms the limitations set to obtain financial records of organized crime beneficiaries (Delfino 2025b). The added measures should focus on financial crimes, and allow the CCSS to prohibit or severely restrict individuals, who owe large debts to the Fund, from accessing more debt without scheduling repayments to their previous financial responsibilities. This reform would force employers that abuse LLC structures to avoid paying the institution, an example of this being the largest corporate tax-evader of the CCSS, which owed (as of 2018), more than ₡2 billion CRC to the institution, but was not forced to pay since current laws only allow the fund to close companies that do not pay. Using the proposed reforms to investments, banking, institutions, and debt repayment, the State can address the root cause of the sector's financial woes, so that it can continue protecting and serving all Costa Rican citizens.

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Annex

1. Former Minister of Finance, during the Jimenez Oreamuno administration of 1924-1928, and then-Manager of the National Bank of Insurances.
2. Then-congressman of the President's party, elected in 1938.
3. Former Minister of Government, during the Gonzalez Viquez administration of 1928-1932, and then-Manager of the National Bank of Costa Rica. Known for censoring Communist media publications during his time as Minister (Cruz 1980).
4. Then-congressman of the President's party, originally elected in 1932. The changes made were born from the views of Soley Güell, which he also shared. Due to his ideological disagreements with Dr. Padilla Castro, he recommended President Calderon Guardia to choose someone other than Dr. Padilla Castro to serve as Manager of the CCSS, and eventually former President Julio Acosta Garcia was chosen, but Dr. Padilla Castro remained in the institution as the Assistant Manager, working under Acosta Garcia (de la Cruz 1996) & (Mujer y Hogar 1945).
5. First known as the Polyclinic, then as the Central Hospital, and nowadays as the Dr. Rafael Angel Calderon Guardia Hospital, still functioning in downtown San José, as of December 2025.
6. The Communist Party was officially dissolved, but immediately refounded with the new name. This change was done to obtain the support of Archbishop Victor Sanabria, the highest ranking member of the Catholic Church in Costa Rica. In exchange for this, Archbishop Sanabria allowed the PVP to spread political propaganda amongst the Catholic masses of the country. This formalized the new triple alliance in Costa Rican politics between the Communists, the Church, and the Republican Party (Aguilar 1986).
7. The authoritarian threat was former Calderon Guardia ally, Nazi supporter, and president, Leon Cortes Castro. Extensive evidence exists of his ties to Nazi Germany:
 - a. The President of the Nazi Party of Costa Rica, Max Effinger, was his Director of Immigration and Minister of Public Works and Transportation throughout his entire presidency (1936-1940)
 - b. Correspondence between him and Adolf Hitler exists, where Hitler thanks him for sending Dr. Alfonso Acosta Guzmán to a medical conference held in Germany in 1936.
 - c. He also hosted the Nazis aboard the German Ship SMS Schleswig-Holstein between January 20-29, 1937. He went aboard the ship and allowed navy training exercises to be performed off the coast of Limon. This ship is notable, given that the first cannon shots of World War II were fired by it (Wikipedia 2025) & (La Tribuna 1937)
 - d. A document from the Ministry of Foreign Relations, dated November 7th, 1939 ordered all Costa Rican consulates in the world to deny all visas to Jewish people, stating that "this prohibition is general, and by no means will exceptions be granted on this procedure" (Chevez Hidalgo 2015) & (La Teja 2020)
8. The Doctors Association had promised not to interfere with the functioning of the CCSS when it was founded, even though they opposed socialized medicine. A doctor explained this position, stating that "Social security, as a collective service, stifles the private initiative of the physician. While it may be beneficial for lower-income social classes,

from the doctor's perspective, it is detrimental to higher-income groups who can afford private medical services, which is why [doctors] do not agree with it covering the entire population."

9. Otilio Ulate Blanco was the owner of the newspaper *Diario de Costa Rica*, which he shut down in protest of what he perceived as Calderon's authoritarianism, but reopened it shortly after the 1944 election. He was also a former Congressman for the province of Alajuela. His coalition contained many important politicians in Costa Rica's history, including Jose Figueres Ferrer, founder of the National Liberation Party (*La Tribuna* 1944).
10. For the 1944 election, President Calderon Guardia's Republican Party and the Popular Vanguard Party (PVP, the new name of the Communist Party of Costa Rica⁶), signed a coalition agreement to form the "Victory Bloc". This bloc agreed to only support the presidential candidacy of the Republican Party's candidate Teodoro Picado Michalski, to "once and for all defeat all sources of authoritarianism in the country⁷" (*Pacto Bloque de la Victoria* 1943). Picado Michalski is eventually elected with a margin of victory of 32% (*La Tribuna* 1944). The Victory Bloc's resounding victory allowed for President Picado Michalski to continue the labor of his predecessor in strengthening the CCSS, among other institutions.
11. Accounts of what happened that evening are contradictory, the police claim that those inside the house fired at them after they tried searching the house for firearms, while those inside said that, after the police surrounded the house, Dr. Valverde Vega walked out with a white cloth in hand, and the police shot him without due cause (*Espiritu del 48*, n.d).
12. The three main reforms instituted by the Social Warranties include the creation of the CCSS, the Labor Code, and the University of Costa Rica. All of which are still integral parts of Costa Rican society as of 2025.
13. If more funds were to be needed, the law states it is the duty of the Executive power to account for this deficit in next year's budget (*de la Cruz* 1995a).
14. Article 1: The CCSS is hereby allowed to invest the reserves of its IVM regime in mortgage loans at an 8% annual interest rate. When it comes to agricultural or cattle-raising loans, the type of interest cannot be greater than a 6% annual interest rate. Article 2: Additional to article 39 of [Law N° 17 of 1941], under equal conditions, employers and workers that contribute to the IVM regime will have preference when requesting credit under the system established by the mentioned institution (CCSS 1962).
15. Originally a private bank created with British and Costa Rican capital in 1863, but was nationalized in 1949 by the Founding Board of the Second Republic, when it chose to nationalize all banks in Costa Rica.
16. The ceiling was progressively raised with changes done twice for 4 years, so that in October of 1975 the ceiling would be completely eliminated.
17. This included hospitals controlled by the Ministry of Health, the Boards of Social Protection, and those owned by UFCo. All takeovers could not require payment from the CCSS, and all workers kept their labor benefits from years served at these institutions. No

syndicates or unions liquidated their benefits thanks to honest and direct communication from the heads of the CCSS.

18. This was formally included in the Planification Law of March 1974.
19. This new program was funded using 5% of the CCSS's total IVM revenue from employers, and 20% of the Central Government's total sales tax revenue. Within two years, almost 60% of these funds were used for school dining programs, drinking water initiatives, and Materno-Infant nutrition programs (Ley N° 5662) & (de la Cruz 1995a).
20. Position created in April 1974, named by the Executive power, and is the highest authority within the CCSS. This position was created in all of the autonomous institutions of the Costa Rican public sector.
21. The system did not take in the funds that the IVM had, instead the IVM contributors that were above 35 years of age, and retirees, were kept in the same system now renamed SEBD, while contributors under the age of 35 were given the option to switch to the Personal Accounts subsystem, for which they had 2 years to decide or be locked into SEBD. All new contributors after January 1st 2008 were placed in the Personal Accounts subsystem (Ley N° 51 de 2005).
22. Both are Public Banks of the Panamanian State.
23. Introduced by Congresswoman Rocio Alfaro Saenz, the minority leader of the Broad Front Party (FA). It is highly likely that this bill won't be discussed by the 2022-2026 Congress, so it should be prioritized by the 2026-2030 Congress.
24. A mechanism known as the "Extraordinary Sessions", where the Executive Power can prioritise discussions in Congress of its policy agenda for 3 months.
25. Named after Dr. Max Peralta Jimenez, graduate of the University of Pennsylvania's Med School in 1896. He is remembered by his selfless decision to stop looking for cures to his cancer to serve the residents of Cartago as their Town Doctor. This delay has caused the current hospital's ER to operate at 160% capacity, and forcing its administrators to rent 11 additional building in distinct points of the province at an annual cost of ₡263 million CRC, while the Board of Directors of the CCSS continued to delay, resulting in developers charging an additional \$85 million USD due to the initial agreement expiring as a consequence of the delay (Diario Extra 2025) & (EcuRed 2017).
26. Introduced by Congresswoman Sofia Guillen Perez of the FA. Refer to Annex #22 for the timeline of processing.